

KAAF UNIVERSITY, FETE KAKRABA.

DEPARTMENT OF NURSING

END OF SECOND SEMESTER EXAMINATION, 2024/2025

BSC NURSING LEVEL 200 - REGULAR

KNUR 216 (CREDITS 3): SURGICAL NURSING 2

TIME ALLOWED:

INSTRUCTIONS: Carefully read the questions and answer all questions.

CIRCLE THE CORRECT ANSWER.

1. What is the primary mechanism by which atherosclerosis leads to coronary artery disease?
 - A. Gradual and plaque formation.
 - B. Hypertension and cardiac remodeling.
 - C. Thrombosis and embolism.
 - D. Vasoconstriction and vasodilation.

ANSWER: A

2. A patient with intestinal obstruction is at risk for developing these long-term complications **EXCEPT**:
 - A. Intestinal failure.
 - B. Keloid formation
 - C. Malabsorption.
 - D. Short bowel syndrome.

ANSWER: B

3. What is the term characterized by twisting of the intestines leading to obstruction and ischemia?
 - A. Adhesion.
 - B. Hernia.
 - C. Intussusception.
 - D. Volvulus.

ANSWER: D

4. The following are the main indications for surgical intervention in intestinal obstruction **EXCEPT**?
- A. Absence of complications.
 - B. Complete obstruction.
 - C. Failure to conservative management.
 - D. Prescence of complications (perforation, ischemia).

ANSWER: A

5. What is the most accurate imaging modality for a patient suspected of intestinal obstruction?
- A. Computed tomography (CT) scan.
 - B. Family history.
 - C. Physical evaluation.
 - D. Plain abdominal x- ray.

ANSWER: A

6. What is the term for the surgical intervention on a patient with intestinal obstruction that involves the removal of the affected segment of the intestines and anastomosing the remaining healthy intestines?
- A. Exploratory laparotomy.
 - B. Intestinal bypass
 - C. Ostomy.
 - D. Resection and anastomosis.

ANSWER: D

7. The most common complication of untreated typhoid fever is called:
- A. Cellulitis
 - B. Hemorrhage.
 - C. Inflammation.
 - D. Intestinal perforation.

ANSWER: D

8. The main aim of initial management of intestinal obstruction is to:
- A. Administer antibiotics.

- B. Correct fluid and electrolyte imbalance.
- C. Perform surgery.
- D. Relieve the obstruction.

ANSWER: B

9. A patient reported with sudden abdominal pain, nausea, vomiting, and constipation. What is the most likely diagnosis to confirm the condition?
- A. Gastroenteritis.
 - B. Inflammatory bowel disease.
 - C. Intestinal obstruction.
 - D. Irritable bowel syndrome.

ANSWER: C

10. What is the most common complication of small bowel obstruction?
- A. Adhesions.
 - B. Hernias.
 - C. Intussusception.
 - D. Tumors.

ANSWER: C

11. A patient with coronary artery disease (CAD) is advised to make lifestyle modifications. What is the main goal of this lifestyle modification in CAD?
- A. To improve blood flow to the myocardium.
 - B. To lower cholesterol level.
 - C. To reduce blood pressure.
 - D. To reduce cardiovascular risk.

ANSWER: D

12. What is the term for the procedure that involves insertion of a balloon to widen a narrowed coronary artery?
- A. Angioplasty.
 - B. Atherectomy.
 - C. Stenting.
 - D. Thrombectomy.

ANSWER: A

13. A patient reported at your facility with the diagnosis of coronary artery disease. What is the most likely cause of coronary artery disease?

- A. Atherosclerosis.
- B. Diabetes mellitus.
- C. Hyperlipidemia.
- D. Hypertension.

ANSWER: A

14. A patient was diagnosed of esophageal atresia and undergoes surgical repair. What is the most likely postoperative complications?

- A. Anastomotic leak.
- B. Gastroesophageal reflux.
- C. Respiratory distress.
- D. Stricture formation.

ANSWER: A

15. A patient with esophageal atresia had gastrostomy done. What is the role of gastrostomy in the management of esophageal atresia is to?

- A. Decompress the stomach.
- B. Facilitate esophageal anastomosis
- C. Prevent aspiration
- D. Provide nutrition

ANSWER: D

16. A patient with esophageal atresia and not promptly managed is at risk of developing the following **EXCEPT**?

- A. Aspiration pneumonia
- B. Gastroesophageal reflux disease.
- C. Pulmonary embolism.
- D. Respiratory distress.

ANSWER: C

17. A new born infant was brought to the health facility with excessive drooling, coughing, and choking during feeding is likely to be diagnosed of:

- A. Esophageal atresia

- B. Gastroesophageal reflux disease (GERD).
- C. Pyloric stenosis.
- D. Tracheoesophageal fistula.

ANSWER: A

18. What is the most common congenital anomaly of the esophagus that is normally reported at health facilities?

- A. Esophageal atresia.
- B. Esophageal dilatation
- C. Esophageal duplication.
- D. Tracheoesophageal fistula.

ANSWER: A

19. What is the purpose of the tracheostomy tube ties in the management of tracheostomy care? It is to:

- A. Administer medication.
- B. Inflate the tracheostomy cuff.
- C. Monitor oxygen saturation
- D. Secure the tracheostomy tube in place.

ANSWER: D

20. The main aim of tracheostomy care is to:

- A. Facilitate communication.
- B. Maintain patency of the airway.
- C. Prevent infection.
- D. Promote healing.

ANSWER: B

21. What is the recommended frequency for changing the inner cannula of the tracheostomy tube?

- A. As needed.
- B. Every 8 hours.
- C. Every 12 hours.
- D. Every 24 hours.

ANSWER: D

22. What is the main purpose of tracheostomy tube suctioning? It is to:

- A. Administer medication.
- B. Inflate the tracheostomy tube cuff.
- C. Monitor oxygen saturation.
- D. Remove excess mucus and secretions.

ANSWER: D

23. A patient with esophageal stricture where prompt intervention was not taken is at risk of developing the following complication **EXCEPT**?

- A. Alkaline deficiency.
- B. Mineral deficiency.
- C. Protein deficiency.
- D. Vitamin deficiency.

ANSWER: A

24. What is the term for the surgical intervention that involves the removal of the affected segment of the esophagus and re-anastomosing the remaining esophagus?

- A. Esophagectomy.
- B. Esophago-gastrotomy.
- C. Esophago-myotomy
- D. Gastrotomy.

ANSWER: A

25. What is the most appropriate surgical intervention for typhoid perforation?

- A. Laparotomy and simple closure of a complicated perforation.
- B. Laparotomy and resection of the affected bowel segment.
- C. Simple closure of a complicated perforation.
- D. Resection of the affected gangrenous bowel segment.

ANSWER: B

26. What is the main reason for the administration of proton pump inhibitors (PPIs) in the management of esophageal stricture?

- A. To prevent recurrence of stricture.
- B. To promote healing.

- C. To reduce inflammation.
- D. To treat underlying gastroesophageal reflux disease (GERD).

ANSWER: C

27. Pleural effusion is a collection of fluid in the pleural cavity. In empyema, what is collected in the pleural cavity?

- A. Blood.
- B. Lymphatic fluid.
- C. Pus.
- D. Serous fluid.

ANSWER: B

28. The main aim in performing the procedure of dilatation in esophageal stricture is to:

- A. Remove stricture.
- B. Treat the stricture.
- C. Treat the underlying gastroesophageal reflux disease (GERD).
- D. Widen the structure.

ANSWER: D

29. The main diagnostic investigation for a patient with suspected esophageal stricture is:

- A. Barium swallow.
- B. CT scan.
- C. Esophago-gastroduodenoscopy (EGD).
- D. Endoscopy ultrasound.

ANSWER: C

30. A patient presented to the health facility with a progressive dysphagia to solids then to liquids. Which of the following is **NOT** a characteristic feature of esophageal stricture?

- A. Obesity.
- B. Painful swallowing (odynophagia).
- C. Regurgitation of food.

D. Weight loss.

ANSWER: A

31. A patient was diagnosed of esophageal stricture. What is the most common cause of esophageal stricture?

- A. Caustic ingestion.
- B. Esophageal cancer.
- C. Gastroesophageal reflux disease (GERD).
- D. Radiation therapy.

ANSWER: B

32. What is the term whereby part of the intestine twists around itself with its supporting mesentery?

- A. Complicated diverticulitis
- B. Diverticulosis.
- C. Diverticular disease.
- D. Volvulus.

ANSWER: D

33. What is the main aim of postoperative care in general surgical intervention? It is to:

- A. Alleviate pain completely.
- B. Prevent infection.
- C. Promote wound healing.
- D. Reduce pain to a tolerable level.

ANSWER: D

34. A patient reported at the facility with volvulus, what is the direction of the volvulus?

- A. Clock wise.
- B. Counter clockwise / anti clock wise.
- C. Moves alongside the mesentery.
- D. Moves together with the mesentery.

ANSWER: B

35. A patient was diagnosed with sigmoid volvulus. The following is **NOT** a type of volvulus:

- A. Cardiac volvulus.
- B. Cecal volvulus.
- C. Gastric volvulus.
- D. Sigmoid volvulus

ANSWER: A

36. A 60-year-old man reported at the health facility with severe lower left quadrant abdominal pain, fever, and leukocytosis. What is the most possible diagnosis?

- A. Appendicitis.
- B. Cholecystitis.
- C. Pancreatitis.
- D. Volvulus.

ANSWER: D

37. A patient with suspected achalasia. What is the primary diagnostic test for achalasia?

- A. Barium swallow.
- B. Esophagogastroduodenoscopy (EGD).
- C. Esophageal manometry.
- D. Upper GI series.

ANSWER: C

38. A patient with perforated peptic ulcer had delayed surgical intervention is at risk for developing the following condition **EXCEPT**:

- A. Abscess.
- B. Fistula.
- C. Granulation.
- D. Wound infection.

ANSWER: C

39. What is the term of the condition characterized by inflammation of the peritoneum which can occur as a complication of perforated peptic ulcer?

- A. Abscess.
- B. Fistula.
- C. Peritonitis.
- D. Sepsis.

ANSWER: C

40. A patient with perforated peptic ulcer is at risk of developing the following electrolyte imbalance **except**:

- A. Hypocalcemia.
- B. Hyperglycemia.
- C. Hypokalemia.
- D. Hyponatremia.

ANSWER: B

41. A patient reported with suspected perforated peptic ulcer. What is the most common location of perforated peptic ulcer?

- A. Greater curvature of the stomach.
- B. Lesser curvature of the jejunum.
- C. Lesser curvature of the stomach.
- D. Lower part of the esophagus.

ANSWER: A

42. A patient reported to the health facility with the diagnosis of perforated peptic ulcer, and requires fluid resuscitation. What is the aim of fluid resuscitation in this setting? To:

- A. Evaluate cardiac output.
- B. Improve electrolyte imbalance.
- C. Prevent acute kidney injury.
- D. Restore blood pressure (volume).

ANSWER: C

43. What is the main treatment (surgical intervention) for perforated peptic ulcer?

- A. Conservative management with fluid resuscitation and observation.
- B. Endoscopy treatment with clips or glue.

- C. Medical management with proton pump inhibitors (PPIs).
- D. Surgical repair of the perforation.

ANSWER: D

44. What is the most common cause of perforated peptic ulcer?
- A. Helicobacter pylori infection.
 - B. Non-steroidal anti-inflammatory drugs (NSAID) use.
 - C. Stress.
 - D. Zollinger-Ellison syndrome.

ANSWER: A

45. The main pathophysiological mechanism underlying achalasia?
- A. Abnormal peristalsis in the esophageal body.
 - B. Decrease muscle tone in the lower esophageal sphincter.
 - C. Increase muscle tone in the lower esophageal sphincter (LES).
 - D. Failure of the lower esophageal sphincter to relax during swallowing.

ANSWER: D

46. What is the term given to the condition characterized by inflammation of the peritoneum, which can occur as a complication of typhoid perforation?
- A. Abscess.
 - B. Fistula.
 - C. Peritonitis.
 - D. Sepsis.

ANSWER: C

47. A patient with perforated peptic ulcer requires fluid resuscitation. What type of fluid that must be administered in this setting?
- A. Blood products.
 - B. Colloids (blood products).
 - C. Crystalloids (iv fluids).
 - D. Electrolyte-rich fluids

ANSWER: C

48. A patient with sigmoid volvulus is at risk for developing which of the following potential complications?

- A. Colorectal cancer.
- B. Inflammatory bowel disease.
- C. Intestinal infarction.
- D. Irritable bowel syndrome.

ANSWER: C

49. A patient with esophageal stricture is at risk of developing the following complication, **EXCEPT**?

- A. Bleeding.
- B. Esophageal perforation.
- C. Malunion.
- D. Recurrence of stricture.

ANSWER: C

50. What is the long-term outcome for patients with esophageal atresia?

- A. Chronic esophageal dysfunction.
- B. Increase risk of esophageal cancer.
- C. Normal esophageal function.
- D. Normal growth and development.

ANSWER: A

51. What is the main cause of coronary artery disease (CAD)?

- A. Atherosclerosis.
- B. Diabetes mellitus.
- C. Hyperlipidemia.
- D. Hypertension.

ANSWER: A

52. Which diagnostic test uses x-rays and contrast to visualize the coronary arteries?

- A. Cardiac catheterization.
- B. Coronary angiography.
- C. Echocardiography.

D. Stress test.

ANSWER: B

53. A patient with intestinal obstruction is at risk for developing the following complication, **EXCEPT**?

- A. Dehydration.
- B. Electrolyte imbalance.
- C. Generalized edema.
- D. Sepsis.

ANSWER: C

54. What is the definition of empyema?

- A. Accumulation of fluid in the pleural space.
- B. Accumulation of pus in the pleural space.
- C. Infection of the lung parenchyma.
- D. Inflammation of the pleura.

ANSWER: B

55. A 45-year-old man reported at the health facility with fever, chest pain, and shortness of breath, what is the most likely diagnosis?

- A. Asthmatic attack.
- B. Empyema.
- C. Malaria.
- D. Patent ductus arthrosis.

ANSWER: B

56. A patient diagnosed of empyema underwent diagnostic thoracentesis. What is the possible finding of the fluid?

- A. Bloody fluid.
- B. Clear or straw-colored fluid.
- C. Cloudy or turbid fluid (pus).
- D. Serous fluid.

ANSWER: B

57. What is the term for the surgical intervention of empyema?

- A. Decortication.
- B. Pleurectomy.
- C. Thoracotomy.
- D. Video- assisted thoracic surgery (VATS).

ANSWER: A

58. Which of the following is at risk factor for developing cholelithiasis?

- A. Age < 40 years.
- B. High dietary fiber intake.
- C. Obesity.
- D. Male sex.

ANSWER: C

59. The following is **NOT** a contraindication for laparoscopic cholecystectomy:

- A. Cholelithiasis.
- B. Pregnancy.
- C. Previous abdominal surgery.
- D. Severe cholecystitis.

ANSWER: A

60. What is the most common complication of untreated typhoid perforation?

- A. Bleeding.
- B. Hemorrhage.
- C. Intestinal perforation.
- D. Meningitis.

ANSWER: C

61. The following antibiotics are used after the surgical intervention of typhoid perforation **EXCEPT**?

- A. Acetaminophen.
- B. Azithromycin.
- C. Ceftriaxone.
- D. Ciprofloxacin.

ANSWER: A

62. Which of the following is **NOT** characteristic feature of typhoid perforation?

- A. Absence of rebound tenderness.
- B. Gradual onset of the abdominal pain.
- C. Presence of rebound tenderness.
- D. Sudden onset of severe abdominal pain.

ANSWER: A

63. What is the common cause of aortic stenosis in adults?

- A. Calcific aortic stenosis.
- B. Congenital bicuspid aortic valve.
- C. Endocarditis.
- D. Rheumatic heart disease.

ANSWER: A

64. Which of the following is a contraindication for air enema reduction in intussusception **EXCEPT**?

- A. Absence of peritonitis.
- B. Free air in the abdomen.
- C. Presence of peritonitis.
- D. Severe abdominal distension.

ANSWER: A

65. What is the most common complication of untreated intussusception?

- A. Bowel perforation.
- B. Bowel obstruction.
- C. Peritonitis.
- D. Sepsis.

ANSWER: A

66. What is the diagnostic test of choice for evaluating the severity of aortic stenosis?

- A. Cardiac catheterization
- B. Chest x-ray.
- C. Transesophageal echocardiogram (TEE).

D. Transthoracic echocardiogram (TTE).

ANSWER A

67. Which of the following surgical procedures is used to correct tricuspid stenosis?

- A. Aortic valve replacement.
- B. Mitral valve repair.
- C. Mitral valve replacement.
- D. Tricuspid valve replacement.

ANSWER: D

68. Which of the following valves is most commonly affected by rheumatic heart disease?

- A: Aortic valve.
- B: Mitral valve.
- C: Pulmonary valve.
- D: Tricuspid valve.

ANSWER: B

69. The surgical procedure of choice for severe aortic stenosis in patients who cannot withstand open- heart surgery?

- A. Balloon valvuloplasty.
- B. Mitral valve repair.
- C. Open - heart surgery with aortic valve replacement.
- D. Transcatheter aortic valve replacement (TAVR).

ANSWER: D

70. What is the main surgical intervention in the treatment of Hirschsprung's disease?

- A. To create a permanent colostomy.
- B. To remove the affected segment of the colon.
- C. To repair the anal sphincter.
- D. To restore normal bowel function by removing the aganglionic segment.

ANSWER: D

71. The distinct indication of intra operative pleural irrigation in empyema surgery is the following **EXCEPT**?

- A. To improve lung expansion.
- B. To reduce pain postoperatively.
- C. To reduce the risk of complications.
- D. To reduce the risk of recurrence.

ANSWER: B

72. The following is **NOT** a post operative strategy for patients undergoing empyema surgery:

- A. Early ambulation
- B. Nutritional status.
- C. Pain control.
- D. Respiratory physiotherapy.

ANSWER: B

73. The following is **NOT** a potential complication of surgical intervention in empyema:

- A. Bronchopleural fistula.
- B. Chronic renal failure.
- C. Empyema recurrence.
- D. Respiratory failure.

ANSWER: B

74. A 35-year-old woman was diagnosed with empyema on the right lung.

What is the main aim of the surgical intervention in empyema? To:

- A. Decorticate the affected lung.
- B. Drain the empyema cavity.
- C. Remove the underlying cause of empyema.
- D. Repair the diaphragm.

ANSWER: B

75. A malnourished woman underwent resection and anastomosis of typhoid perforation. She is likely to develop the following postoperative complications **EXCEPT**?

- A. Enteric fistula.
- B. Intra- abdominal abscess.
- C. Keloid formation.
- D. Wound infection.

ANSWERS: C

76. What is the most appropriate surgical intervention for typhoid perforation?

- A. Exploratory laparotomy
- B. Resection of the affected intestines.
- C. Right hemicolectomy.
- D. Simple closure of the perforation.

ANSWER: A

77. What is the term for the surgical intervention for the condition pleural effusion?

- A. Decortication.
- B. Incision and drainage.
- C. Thoracentesis.
- D. Thoracotomy.

ANSWER: C

78. The Hirschsprung's disease can conveniently be classified under two main groups?

- A. Duration and location.
- B. Extent and location.
- C. Plan and duration.
- D. Plan and extent.

ANSWER: B

79.The diagnosis of the condition Hirschsprung's disease involves a combination of clinical evaluation, imaging and lab investigation. What lab investigation is mainly done?

- A. Blood urea.
- B. Full blood count.
- C. Kidney function test.
- D. Rectal biopsy.

ANSWER: D

80.The following surgical intervention can be performed on a patient with the condition Hirschsprung's disease **EXCEPT**?

- A. Colostomy (temporal).
- B. Laparoscopic surgery.
- C. Pull- Through procedure.
- D. Resection and anastomosis.

ANSWER: B

81.The following is **NOT** a complication of Hirschsprung's disease:

- A. Enteritis.
- B. Ileus.
- C. Toxic megacolon.
- D. Malunion.

ANSWER: B

82.A patient reported at the health facility with suspected Deep Vein Thrombosis, the following diagnostic investigations are to be carried out **EXCEPT**?

- A. Computed tomography (CT) scan.
- B. Magnetic resonance imaging (MRI).
- C. Venography.
- D. X - ray.

ANSWERS: D

83.The following is **NOT** a postoperative care on the drainage tube inserted in pleural effusion:

- A. Care of the wound site.
- B. Drainage of the fluid.
- C. Follow-up visits.
- D. Proper position of the tube.

ANSWER: C

84. What are the three main causes of Deep Vein Thrombosis (DVT).
- A. Blood coagulation disorders, damage to blood vessel walls and immobility.
 - B. Blood disorders, immobility and long periods of surgeries.
 - C. Damage to blood vessel walls, long period of surgeries and immobility.
 - D. Immobility, coagulation disorders and long periods of surgeries.

ANSWER: A

85. A child very dehydrated and critically ill was brought to the health facility with the diagnosis of volvulus, an initial surgical intervention was done, what was the term for the initial surgical intervention?
- A. Permanent colostomy.
 - B. Pull – through procedure.
 - C. Resection and anastomosis.
 - D. Temporal colostomy.

ANSWER: D

86. A 35-year-old woman reported at the health facility and was diagnosed of Deep Vein Thrombosis, as a nurse what type of device will you use to rate the patient's pain level in your assessment?
- A. Clam organ assessment chart.
 - B. Numerical rating scale.
 - C. 24 Hour report book.
 - D. Wong barker's assessment scale.

ANSWER: B

87. A patient reported with chronic constipation with sudden, severe pain at the left lower quadrant. What investigation must be done to confirm the diagnosis of suspected volvulus?

- A. CT scan.
- B. Family history.
- C. Only chief complaints.
- D. Plain X- Ray.

ANSWER: D

88.A patient reported with pain in the left arm that radiates to the neck, jaw and throat. What kind of pain is in coronary artery disease?

- A. Angina.
- B. Easily fatigability.
- C. Physical exhaustion.
- D. Referred pain.

ANSWER: A

89.The surgical interventions of valvular stenosis can either be repair of the valve or replace of the valve. What is the term where the mitral valve is replaced?

- A. Sternotomy: Closed heart with mitral valve replacement.
- B. Sternotomy: Opened heart with mitral valve replacement.
- C. Sternotomy: Thoracotomy with mitral valve replacement
- D. Thoracotomy with mitral valve replacement.

ANSWER: B

90.Among the valves of the heart, which of them does easily become stenosed with infection or calcium deposition?

- A. Aortic valves.
- B. Mitral valves.
- C. Pulmonary valves.
- D. Tricuspid valves.

ANSWER: B

91.The following are indications for tracheostomy to be performed on a patient **EXCEPT**?

- A. To by – pass obstructions from the lower airway.
- B. To facilitate mechanical ventilation.

- C. To maintain patent airway.
- D. To remove secretions from the lungs.

ANSWER: C

92.The following is **NOT** a possible complication after surgical intervention of volvulus:

- A. Anastomotic leak.
- B. Fecal fistula.
- C. Granulation.
- D. Wound infection.

ANSWER: C

93.What are the stages (sequentially) in the condition empyema?

- A. Exudative stage, fibrin purulent stage and organizing stage.
- B. Fibrin purulent stage, organizing stage and exudative stage
- C. Fibrinolytic stage, organizing stage and exudating stage.
- D. Organizing stage, exudative stage and fibrin purulent stage.

ANSWER: A

94.In the condition of coronary artery disease, the coronary arteries are blood vessels of which organ?

- A. The brain.
- B. The heart.
- C. The kidneys.
- D. The lungs.

ANSWER: B

95.What are two main types of intestinal obstruction?

- A. Incidental and mechanical intestinal obstruction.
- B. Mechanical and functional intestinal obstruction.
- C. Traumatic and functional intestinal obstruction.
- D. Traumatic and mechanical intestinal obstruction.

ANSWER: B

96. A patient reported at the health facility with a provisional diagnosis of cholelithiasis, what other diagnosis (gold standard) can actually confirm the condition?

- A. Biopsy.
- B. Biopsy and imaging.
- C. Imaging as in CT scan or Ultra sound.
- D. Laboratory investigation.

ANSWER: C

97. There are two types of intestinal obstruction mechanical and functional. What actually happens at the functional intestinal obstruction?

- A. The obstruction occurs as a result of narrowing of the lumen of the intestines from pressure in the intestinal walls as in intussusception.
- B. The obstruction occurs due to impacted fecal content
- C. The obstruction occurs where the intestines fail to propel the content along the bowel as in amyloidosis.
- D. The obstruction occurs where the intestines twist over itself as in diverticulitis.

ANSWER: C

98. A patient report at the health facility with suspected peptic ulcer, the following investigation can be done at the laboratory to detect the H-pyloric bacteria **EXCEPT**?

- A. Blood (serum).
- B. Breath.
- C. Saliva.
- D. Stool.

ANSWER: A

99. Which of the classification (type) of peptic ulcer has the combination of gastric and duodenal ulcer? Is type:

- A. Four only.
- B. One only.
- C. Three only.

D. Two only.

ANSWER D

100. A patient reported at the health facility with suspected DVT, as a nurse started looking out for the signs of DVT. Which of the following is a sign of DVT?

- A. Pain.
- B. Loss of appetite.
- C. Redness.
- D. Weakness or fatigue.

ANSWERS: C

